



Lynn, 13, receives polio and DPT vaccinations at Burj Hammoud Public School in Beirut. UNICEF, with the Ministry of Public Health, supports routine immunization and essential health services in collective shelters for displaced and vulnerable families.

Highlights

- At least 2,196 people have been killed, including 172 children, and 7,185 wounded, including 661 children, following weeks of deadly airstrikes and military operations in Lebanon.
- UNICEF provided fuel, repair, and maintenance interventions that directly supported the continuity of public water supply and wastewater services for more than 2.8 million people, including an estimated 409,000 displaced people hosted within communities relying on these systems.
- Approximately 11,839 children and caregivers have been supported with community-based psychosocial support in shelters and community spaces, combining psychosocial support with recreational activities to strengthen well-being, resilience, and social cohesion.
- UNICEF requires US\$ 48.3 million to reach one million people in need, yet only US\$ 15.2 million is currently available, leaving a 69 per cent funding gap. Without additional funding, critical life-saving services for children and families, including access to safe water, health and nutrition services, and child protection support will be significantly disrupted.

Situation Overview and Humanitarian Needs

Hostilities across Lebanon continue, with daily airstrikes and continued military operations in southern Lebanon, parts of the Bekaa, and in other areas. Since 2 March, at least 2,196 people have been killed, including 172 children, and 7,185 wounded, including 661 children, across Lebanon according to the Ministry of Public Health (MoPH) as of 16 April.

The humanitarian impact of the conflict remains severe. At least 1.1 million people¹ have been displaced, including 390,000 children, with 141,613 individuals currently sheltering in 690 collective sites.² Many families have experienced repeated displacement and remain unable to return due to ongoing hostilities and widespread destruction. Overcrowding in shelters and sustained pressure on essential services continue, particularly in urban and peri-urban areas. Displacement remains largely outside formal shelter settings, with most households living within host communities, in rented accommodation, or in informal shelters. These conditions limit access to assistance and increase exposure to protection risks.³ In particular, poor living conditions and barriers to humanitarian assistance are heightening risks for women and girls, including exposure to violence, increased unpaid care burdens, and reliance on negative coping strategies, with the most vulnerable groups disproportionately affected. This underscores the need for targeted, gender-responsive support.⁴

¹ Ministry of Social Affairs, April 2026

² Disaster Risk Management Unit, 16 April 2026

³ OCHA Lebanon Flash Update 17, 13 April 2026

⁴ Rapid Gender Analysis - Gendered Impacts of Displacement Outside Collective Shelters in Lebanon, UN Women, March 2026

Lebanon

Humanitarian Flash Update No. 8

Escalation of Hostilities

16 April 2026

unicef 
for every child

Situation in Numbers



172

Children killed since 2 March
(MoPH)



661

Children injured since 2 March
(MoPH)



390,000

Children displaced



1.1 million

People displaced

Civilian infrastructure continues to be impacted, including 18 reported incidents affecting water systems across Baalbek-Hermel, Bekaa, South, and Nabatieh, damaging reservoirs, pumping stations, solar systems, and main supply lines, and disrupting access to safe water. Airstrikes have disabled key crossings, severely disrupting movement and humanitarian access. Access to southern Lebanon has been severely disrupted following the destruction of key transport infrastructure. On 15 April, the Qasmieh bridge, the last remaining crossing linking the Tyre area to Sidon, was destroyed, severing a critical connection between southern Lebanon and the rest of the country. This follows the destruction of four main bridges over the Litani River since 2 March, which has significantly constrained movement and humanitarian access across the south.

Attacks on healthcare and emergency services remain a major concern, placing increasing pressure on an already strained health system. Since 2 March, 93 healthcare workers have been killed and 225 injured, alongside 122 reported attacks on emergency medical services. In total, 109 vehicles and 25 medical facilities have been impacted, and six hospitals have been forced to close.⁵ The impact on service delivery is acute. Tebnine Governmental Hospital, the only operational health facility in Bint Jbeil, had been identified as a high-need facility due to the critical burden on services in the area. UNICEF delivered essential medical supplies to the hospital last week to help sustain services. The facility has since sustained significant damage following repeated airstrikes, including impacts on the emergency department and critical equipment such as ventilators and monitors, further constraining its ability to deliver life-saving care.

Education in Lebanon has been severely disrupted by escalating hostilities and mass displacement. Of the 690 collective shelters, 365 are public schools and 58 are Technical and Vocational Education and Training (TVET) institutions, greatly reducing available learning spaces. In addition, 336 public schools across South and Nabatieh, Bekaa, Baalbek, and Beirut's southern suburbs remain closed due to insecurity, evacuation orders, and proximity to conflict zones. While learning has started in person in safer areas and remotely in schools converted to shelters, the ongoing school closures continue to affect approximately 115,100 students, including 86,700 in the morning shift and 28,400 in the afternoon shift. Further, 16,400 youth have been impacted by TVET school closures. Around 600 schools have resumed learning in safer locations, primarily in North Lebanon and Akkar, including 109 designated hub schools operating double-shift systems to accommodate students from closed or shelter schools. At the TVET level, 55 public schools have resumed in-person learning in safe areas, while 103 schools, either closed or currently used as shelters, have transitioned to online learning delivered by TVET teachers. However, despite these efforts access to education remains uneven, particularly for displaced children and youth facing barriers such as limited connectivity, lack of safe learning spaces, limited access to practical courses, and ongoing insecurity, constraining continuity of learning. Prolonged disruption to education risks increased dropout rates and negative coping mechanisms among children and adolescents, particularly among displaced populations.

UNICEF partnered with the women-led organization FEMALE to conduct rapid assessments of families living in open spaces in Beirut and Saida. Findings indicate that access to shelter remains limited due to safety concerns, lack of privacy, unsuitable conditions, and fear of family separation, with additional accessibility barriers reported. WASH services are constrained, with limited access to toilets and hygiene facilities, insufficient hygiene and dignity kits, and long distances to showers. Education access is extremely limited, as children face difficulties participating in remote learning due to limited electricity and charging capacity. Health services remain under pressure, with mobile medical units requiring extended operating hours.

Summary Analysis of Programme Response

UNICEF is delivering gender responsive and disability inclusive humanitarian programming across Lebanon, including in collective shelters, communities hosting internally displaced persons (IDPs), hotspot areas directly affected by conflict, and hard-to-reach areas south of the Litani River and in the Bekaa, as well as in border areas. The response targets all vulnerable groups, including Lebanese, Syrian refugees, and Palestinian refugees, leveraging funding from the Lebanon Response Plan. UNICEF's programmes combine broad-reach interventions, such as water system operations and maintenance and primary healthcare, with targeted support, including cash transfers to families and individuals and youth engagement initiatives.

Rapid Response Mechanism

As of 16 April, the Rapid Response Mechanism (RRM) has reached 210,350 IDPs in more than 378 shelters, as well as people on the move and in hard-to-reach areas, with essential non-food items (NFI). The second round of RRM distributions in collective shelters, initiated on 7 April, reached approximately 48,580 IDPs. Distributions included 692,921 bottles of water (1.5L each), 43,669 family hygiene kits, 18,570 baby kits, 23,598 dignity kits, 1,982 packs of adult diapers and sanitary pads, and 1,340 boxes of high-energy biscuits for children aged six months and above and pregnant women. Winterization support

⁵ Public Health Emergency Operation Centre Update – 16 April 2026

included 28,466 blankets and 23,598 children's clothing kits. In addition, around 4,900 families on the move in Beirut, Saida, and Tyre received light RRM kits and bottled water.

As part of the RRM response, joint WFP–UNICEF convoys continue to deliver life-sustaining support to children and families in hard-to-reach, high-risk areas despite severe access constraints and ongoing hostilities. Since 2 March, more than 100,000 people in hard-to-reach areas have been reached with life-sustaining UNICEF assistance.

As part of this continued effort, a joint WFP/UNICEF/Logistics Cluster convoy on 14 April successfully reached Hasbaya and seven villages in the Arkoub area (Nabatieh governorate), targeting approximately 2,800 people. The mission delivered essential, life-sustaining support to communities that remain intermittently cut off due to insecurity and damaged infrastructure. In parallel, on 15 April the first UNICEF convoy reached the Rashidye Palestinian camp just a few kilometres from the frontline in Naqoura, where displaced Palestinian, Lebanese, and Syrian families from the south have sought refuge. The mission targeted approximately 3,000 families with essential supplies. A second convoy, planned for 17 April to reach an additional 1,500 families, was cancelled after Tyre became inaccessible following the destruction of the only remaining bridge.

Additional joint convoys planned for this week have been put on hold due to ongoing insecurity and access constraints, underscoring a highly volatile operating environment. Despite these challenges, UNICEF continues to adapt its delivery modalities to reach children and vulnerable families whenever access becomes feasible, prioritising hard-to-reach areas through a combination of convoys and prepositioning while sustaining life-saving support and identifying critical service gaps to inform a more continuous, child-centred response.

Water, sanitation and hygiene

In coordination with the Ministry of Energy and Water (MoEW), UNICEF supplied over 365,000 litres of fuel to Water Establishments, supporting the operation of approximately 72 water facilities, including water pumping stations, and 34 wastewater stations, including lifting stations nationwide. In parallel, UNICEF undertook critical repair and maintenance works, including 50 repairs to water systems, primarily water pumping stations and associated networks, and 9 repairs to wastewater systems. These fuel, repair, and maintenance interventions directly supported the continuity of public water supply and wastewater services for more than 2,800,000 people, including an estimated 409,000 IDPs hosted within communities relying on these systems.

UNICEF partners are delivering emergency WASH assistance in 289 collective shelters across Lebanon, reaching an estimated 60,641 IDPs, alongside the distribution of essential hygiene items through the RRM. Support in collective shelters includes repairs to existing WASH facilities and the installation of new showers and latrines, including disability-friendly and gender responsive units. In addition, 306 centre cleaning kits were distributed to support safe and hygienic shelter operations.

Furthermore, 16,920 cubic metres of water was provided through water trucking, reaching 33,300 IDPs, while 448 cubic metres of sludge was safely removed through desludging operations in shelters hosting displaced populations. Rapid WASH technical assessments have been completed in the majority of shelters to identify evolving needs, including urgent diesel requirements to sustain critical water and wastewater pumping stations. Displacement locations are being mapped to assess connectivity to existing water networks and determine the need for water trucking or alternative supply options. Wastewater pumping stations continue to be closely monitored to prevent overflows and mitigate associated public health risks.

Child protection and GBViE

Across the response, 196 shelters and 91 locations were reached through 17 partners (16 operating inside shelters and 1 outside), supporting 27,899 individuals, including 15,035 in IDP collective sites and 12,864 in host communities, through both child protection and GBV services and interventions.

Community-based psychosocial support (CBPSS) reached 11,839 children and caregivers through structured sessions in shelters and community spaces, combining psychosocial support with recreational activities to strengthen well-being, resilience, and social cohesion. Caregivers also received practical guidance on stress management, communication with children, key protection risks, and available services and referral pathways. Case management services were provided to 36 children at risk or survivors of violence. In parallel, 43 unaccompanied and separated children were identified, including 16 as a result of the escalation on 8 April. To date, 35 children have been reunited with their caregivers or placed in appropriate community-based care.

Child protection messaging reached 21,968 individuals through direct outreach and social media, promoting safer practices and strengthening families' ability to protect children. Awareness materials on child separation during conflict were disseminated in approximately 60 per cent of collective shelters and across more than 40 public and private hospitals nationwide.

The digital explosive ordnance risk education (EORE) campaign reached over 1.5 million individuals across multiple platforms, including in hard-to-reach areas. In addition, 776 frontline workers were reached through online sensitization sessions, and 46,978 Information, Education, and Communication (IEC) materials were distributed across 40 shelters and communities in the South, West Bekaa, and Baalbek-Hermel.

Mitigation of heightened gender-based violence (GBV) risks for women and girls, driven by displacement, overcrowding, and lack of privacy in shelters, continues through targeted interventions. Since the beginning of the conflict, 3,244 dignity kits (551 inside collective sites and 2,693 outside) have been distributed to women and girls in Akkar, North, and Bekaa, representing approximately one-third of the GBV sector reach. In addition, 850 GBV awareness sessions (755 inside collective sites and 99 outside) have been delivered to women and girls in Baalbek El Hermel, Beirut, North, and South. Emergency sessions under the POWER4Girls initiative have also been implemented by Makani Centre⁶ and non-GBV partners, providing adolescent girls with life-saving information and practical skills. Furthermore, nine GBV safety audits have been conducted by POWER4Girls education partners with concrete recommendations being addressed at the shelter level to mitigate GBV risks on displaced women and girls. Despite referrals from MoSA social workers to GBV partners, disclosure remains limited due to lack of trust and non-conducive conditions within collective sites, including overcrowding and lack of privacy and confidentiality. GBV case management continues, primarily through remote modalities, to ensure ongoing support to survivors.

Education

UNICEF continues to support The Ministry of Education and Higher Education's (MEHE) phased response to sustain learning continuity for affected children. So far, 608 schools, mainly in North Lebanon and Akkar, have resumed learning, supporting 141,800 students through UNICEF's Transition and Resilience Education Fund (TREF). This includes 109 hub schools hosting students from other closed or shelter schools on alternating schedules. Further reopenings are expected in Mount Lebanon and Bekaa.

UNICEF is also supporting universal remote learning for all public school students through multiple digital platforms. Accounts and virtual classrooms have been established, reaching 40,000 teachers and 390,000 students. Collaboration with the Ministry of Telecommunications provides up to 20GB of free data per weekday, while monitoring mechanisms on MS Teams and Madristi track participation. So far, 99,794 students are actively using the platforms, representing a 33.7 per cent engagement rate. Teacher participation is significantly higher, with 17,236 teachers engaged (59 per cent), while school-level activation is near universal, with 1,216 out of 1,226 schools active (99 per cent). In total, 40,301 online classes were delivered, amounting to 23,617 hours of instructional time.

The Call and Learn Hotline provided tutoring support to 3,030 children since April 1 through 5,721 calls. The service continues to provide critical academic and psychosocial support to children through both individual and group-based interactions.⁷

Through the Multiple Flexible Pathways (MFP) programme, 975 education kits have been distributed in 174 collective shelters, reaching 30,350 children with items such as learning supplies, recreational supplies, and LEGO playboxes. In addition, 30,452 children are accessing integrated education services across 81 Temporary learning spaces including 32 Makani and 38 Dirasa schools and 11 newly established Guest schools in safe locations. Community outreach efforts by implementing partners have enabled 950 displaced children from nearby shelters and host communities, particularly in Beirut and Mount Lebanon, to participate in Makani-based learning and integrated support activities, with continued work to scale up capacity in all existing and open Makani centres and Dirasa schools in proximity to shelters and catchment areas of displaced children, as well as introducing additional shifts and a rotational schedule to maximise access to learning and integrated services for crisis affected children.

⁶ Makani centres are community-based, multi-service learning and protection hubs that provide vulnerable children and adolescents with integrated support. Services typically include structured and non-formal education (including catch-up classes), psychosocial support, safe spaces for recreation, and access to digital learning resources. They are designed to reach children affected by displacement, poverty, and social exclusion, helping to address both learning gaps and broader wellbeing needs in one accessible location.

⁷ More information on the hotline can be found [here](#).

The Inclusive Education Programme is mobilizing MEHE multidisciplinary teams to support children with disabilities during the current crisis. To ensure seamless support across online, blended, and in-person modalities, approximately 315 paraprofessionals and special educators were trained on the Microsoft Teams and Madristi platforms on 26 and 27 March. Updated terms of reference were issued in March to reflect the specific roles and responsibilities in emergency learning contexts; additionally, standard operating procedures were developed at the beginning of April to reflect the updated role of the multidisciplinary team to support the education response. In the North and Akkar, 32 inclusive schools are now operational with approximately 44 multidisciplinary team members supporting. Support for children with disabilities is also being integrated into MFP activities, including the provision of assistive devices, therapy, and referrals. In shelters, to date, MFP partners have identified 359 children with disabilities and 284 adults with disabilities. Of these, 66 children with disabilities have received assistive devices, 48 have started therapy services, with a total of 70 children with disabilities supported so far. In addition, 79 adults with disabilities have been referred to other service providers. This multi-pronged approach ensures continuity of education, inclusion, and protection for all learners, particularly displaced children and those with disabilities.

Health

Since the escalation of the conflict on 2 March, UNICEF has remained committed to supporting the health response for displaced populations. This includes the delivery of more than 140 tons of essential medications and medical supplies to MoPH primary healthcare centres, Primary Healthcare Satellite Units (PSUs), and hospitals. These supplies have been distributed to health facilities in hard-to-reach areas through UNICEF convoys, including deliveries to Nabatieh, with additional deliveries planned to reach Baalbek and Hermel.

UNICEF has also supported the MoPH to activate mobile health services through its PSU and AaSalameh programmes, delivering primary healthcare services to displaced populations. Currently, 145 primary healthcare centres (PSUs and AaSalameh) cover up to 290 shelters (40 per cent of total shelters supported by MoPH), while also reaching host communities and hard-to-reach areas. To date, PSUs have delivered more than 34,199 primary healthcare services, with 17,903 beneficiaries receiving essential medications.

Health surveillance in shelters indicate that communicable conditions, particularly lice and scabies, remain a persistent issue linked to overcrowding and poor living conditions. In response, UNICEF has supported the MoPH with large quantities of treatment supplies, which have been distributed to all PSUs and AaSalameh centres since the beginning of the crisis and continue to be provided as part of routine service delivery. This is complemented by close collaboration with WASH partners to address underlying conditions, improve treatment compliance, and enhance outcomes.

Through the AaSalameh primary healthcare community outreach programme, 18,748 children have been vaccinated to date. UNICEF also continues to support the MoPH Public Health Emergency Operations Centre (PHEOC) and central medication warehouses, strengthening coordination, logistics, and staffing to ensure uninterrupted supply, proper storage, and distribution of essential medications to primary healthcare centres and governmental hospitals.

Critical care needs are also being addressed. To date, 184 newborns and children have been admitted to intensive care units across 11 governmental hospitals. Under the UNICEF–MoPH Assistance and Care for War Wounded and Affected Children (ACWA) programme, 30 children with conflict-related injuries received life-saving surgical interventions, while 95 injured children and their caregivers received mental health and psychosocial support since 2 March.

Following the 8 April strikes in Beirut, the majority of injured children in Beirut were transferred to specialised medical centres where seven of the most critical cases were managed under the UNICEF-supported ACWA programme to provide life-saving care. In addition, 66 children with non-life-threatening injuries are currently being assessed for further medical interventions and mental health support.

Nutrition

UNICEF expanded its support to nutrition sector partners through the provision of supplies and capacity-building initiatives, increasing coverage to over 470 shelters. This includes 101 shelters supported through 40 UNICEF-supported PSUs delivering an integrated nutrition package, including wasting prevention and treatment services, and 369 shelters reached through UNICEF and nutrition sector partners. Of these, 106 shelters are supported by education partners providing Early Childhood Development (ECD) services.

UNICEF sustained service delivery for host communities and displaced populations outside shelters through Makani centres, nutrition partners, and PSUs. Notably, 11 PSUs are dedicated to serving populations in hard-to-reach areas in Nabatieh and Baalbek-Hermel, ensuring continuity of care.

UNICEF established additional safe spaces to support the growth and development of young children, bringing the total to four Mother–Baby Corners (MBCs) and ECD corners. These spaces provide safe, private, and supportive environments for breastfeeding and early stimulation activities, while ensuring hygienic conditions for infant and young child feeding (IYCF), particularly during emergencies when caregiving conditions are severely disrupted.

To date, a total of 3,289 caregivers of children under five have received integrated IYCF, nutrition, and ECD counselling and messaging. Additionally, 262 caregivers contacted the national IYCF hotline, of whom 176 were referred for specialized support. In parallel, IYCF rapid assessments reached 183 caregivers to identify and refer children requiring specialized services.

Overall, through the multisectoral implementation of nutrition activities, 6,942 children under five and 1,321 pregnant and breastfeeding women (PBW) were screened and assessed, identifying 165 children and 60 women with wasting, all of whom were referred for treatment. To prevent malnutrition among high-risk groups and support adequate diets for young children, UNICEF pre-positioned 262,900 high-energy biscuits (HEBs), 205,107 ready-to-use complementary food (RUCF) jars, and other energy-protein supplements, sufficient to support over 31,863 children under five and PBW for one month. To date, 10,872 children under five have received emergency food rations and micronutrient supplementation, including HEBs, small-quantity lipid-based nutrient supplements (SQ-LNS), and complementary food jars. In addition, 1,106 pregnant and breastfeeding women and adolescent girls received micronutrient supplementation to prevent deficiencies.

A key challenge was identified in a collective shelter where expired infant formula had been distributed by non-nutrition actors. This issue was promptly flagged by the nutrition sector, and immediate corrective action was taken to withdraw and safely dispose of the expired products. The incident highlighted gaps in oversight of breastmilk substitute (BMS) distribution and remains a concern, as donations and distributions occurring outside agreed guidance and without prior consultation with the MoPH and the Nutrition Sector pose risks to optimal infant feeding practices and child health. In response, coordination mechanisms with MoPH and partners have been strengthened to reinforce adherence to established guidelines and improve oversight of BMS provision to prevent similar risks and safeguard infant health.

Social Protection and Humanitarian Cash Assistance

As part of the immediate response to the current escalation and to support beneficiaries of the National Disability Allowance (NDA), UNICEF and the Ministry of Social Affairs (MoSA) advanced the regular cash assistance payment date for 39,000 Lebanese and non-Lebanese NDA beneficiaries nationwide twice, first at the start of March and then April.

Building on previous emergency cash responses in 2024, UNICEF, MoSA, and the ILO have activated emergency financial support for families with persons with disabilities affected by the conflict. This shock-responsive expansion of the NDA has provided urgent cash assistance to offset additional costs related to disability during displacement, including access to healthcare, assistive services, and basic needs. Using established NDA delivery systems and registries, the first phase of this intervention was launched on 11 March and reached approximately 8,000 families (6,000 Lebanese and 2,000 non-Lebanese, or around 33,000 individuals) as of 19 March, with each household receiving a US\$100 grant for households with a child or young person aged 0–19 years living with a disability. A second phase of the shock-responsive expansion of the NDA was initiated on 2 April through an additional allocation of resources. In this second phase, an additional 4,000 families (3,000 Lebanese and 1,000 non-Lebanese, or around 17,000 individuals) with a young person aged 20–27 years living with a disability began progressively receiving a US\$ 100 emergency grant.

The overall target for this emergency support is 45,000 families with a person with a disability from affected areas, to be reached in a phased approach over three months through a one-off cash transfer based on the availability of resources. This programme complements the national Shock-Responsive Social Protection system, also led by the government, ensuring continuity of support and mitigating the immediate financial impact of displacement on vulnerable households. In addition, ongoing efforts to establish coordinated referral pathways for service delivery for persons with disabilities are taking place with various stakeholders, including MoSA, organisations of persons with disabilities, and at the interagency level.

Youth and Adolescent Development

UNICEF continues to engage youth as active contributors to the emergency response, supporting both service delivery and community resilience. To ensure continuity of formal TVET education, UNICEF is supporting the TVET Directorate in implementing its learning continuity plan, in line with the Government's directive to resume learning from 30 March 2026. The plan targets 51,600 TVET students and youth across Lebanon through a combination of online and in-person modalities. To date, 55 public TVET schools have progressively resumed in-person learning in safe areas, while 103 schools, either closed or serving as shelters, have transitioned to online learning delivered by TVET teachers.

UNICEF, in coordination with the TVET Directorate, has established two temporary TVET learning hubs in Mount Lebanon (Dekwaneh and Bir Hassan), currently hosting 5,000 IDPs, to enable blended and online learning for displaced youth and teachers. To date, 250 TVET modules have been digitized and uploaded to the Madristi platform to support learning continuity, with additional modules under development to further expand access.

In parallel, 15 TVET schools functioning as shelters and hosting 8,161 IDPs have received in-kind support from UNICEF, valued at US\$ 50,000, covering urgent needs such as basic supplies, fuel, hygiene materials, and essential items. Non-formal skilling programmes continue in non-affected areas, reaching 635 youth, while delivery in affected locations has shifted to online modalities where feasible.

Youth are also actively contributing to the emergency response through practical action and service delivery. Items produced by youth have been distributed across 29 TVET shelters, including 682 sleeping bags, 500 blankets, 470 clothing kits for Ramadan and Easter, 1,102 spring kits, and 3,744 winter kits. Under work-based learning and income-generation programmes, 79 youth supported food security efforts by producing 106,690 hot meals and 12,455 cold meals in coordination with WFP across four TVET shelter kitchens. In parallel, 48 youth in Baalbek produced and distributed 50,615 hot meals and 22,475 cold meals across 35 shelters. An additional 185 youth completed the production of 1,600 light clothing kits, 500 blankets, 268 pyjamas, and 1,900 sleeping bags. Eight youth barbers also provided hygiene support, offering haircuts to 107 IDPs in two shelters.

Youth engagement in social cohesion and psychosocial support activities continues, with 71 trained youth facilitators delivering sessions and distributing adolescent kits, reaching 833 children and adolescents across 12 TVET shelters.

Adolescent and youth engagement continues to support resilience and social cohesion. Gender-transformative leadership activities for girls are being implemented across 87 shelters and 33 multifunctional platforms, including POWER4Girls emergency sessions, girls' groups, and community initiatives. To date, 1,413 displaced adolescent girls (997 in shelters and 412 in communities) have been reached, while 120 facilitators have been trained to deliver these sessions.

Risk Communication and Community Engagement, Accountability to Affected Populations, Prevention of Sexual Exploitation and Abuse, Gender, and Inclusion

In March 2026, UNICEF call centres received 7,376 calls from IDPs and host communities, providing critical insight into priority needs and access constraints. Requests for information primarily focused on the availability of shelters, including families living on the streets seeking access, as well as requests for cash assistance and other forms of NFI support for IDPs outside shelters. Additional queries included information on the MoSA referral link for IDP registration, child protection and GBV cases at imminent risk requiring urgent referrals, and support for persons with disabilities in shelters. All cases were referred and followed up to ensure timely responses.

Capacity-building efforts have been scaled up to strengthen safeguarding, accountability, and communication with affected populations. UNICEF convened 179 frontline workers from implementing partners across governorates to reinforce Prevention of Sexual Exploitation and Abuse (PSEA) measures, with a focus on community awareness, reporting mechanisms, and engagement with informal leaders during assistance delivery. In parallel, 74 healthcare professionals from mobile clinic teams were trained in collaboration with the MoPH, strengthening their role in communicating free access to services and available feedback mechanisms, including the UNICEF hotline. UNICEF also conducted safeguarding onboarding for 41 WASH company representatives to strengthen measures to prevent and respond to harm in their engagement with affected communities.

Efforts to strengthen inclusion are ongoing. UNICEF, in collaboration with Organizations of Persons with Disabilities and the Emergency Taskforce for People with Disabilities, is leading a national service mapping exercise to improve access to inclusive and specialized services. Gender and disability are mainstreamed across the response through disability-inclusive and gender-responsive health, nutrition, WASH, education, protection, and cash assistance services. These are designed to address the specific needs of women, girls, and persons with disabilities, including safe access, reproductive health, mental

health, and targeted outreach through community- and women-led actors. Coordination with partners supports identification, referrals, and tailored assistance for girls and children with disabilities, including assessments, provision of assistive devices and therapy services, and strengthened disability-inclusive service mapping.

Risk Communication and Community Engagement (RCCE) mechanisms have been activated and expanded to support the response. This includes a communication tree, updated information platforms, and the dissemination of 11 IEC materials across shelters and communities, reaching 20,199 individuals. These materials cover key sectors including education, child protection, mental health and psychosocial support, and public health. Partners are integrating RCCE approaches through schools and community networks, engaging 2,819 individuals in participatory activities. An online chatbot was also shared with 2,071 users through U-Report. Monitoring and adaptation mechanisms are in place to ensure responsiveness to community needs. A social listening dashboard is actively tracking community tensions and misinformation to inform response adjustments, while rapid RCCE assessments are being integrated into the Emergency Rapid Needs Assessment (ERNA).

Palestinian Programme

Essential services are reaching 22,033 IDPs (5,391 families) across UNRWA shelters and host communities in Palestinian camps and gatherings, including populations experiencing repeated displacement.

Integrated multisectoral services, including Makani programming, are delivered through operational centres, reaching 7,813 children with combined education, child protection, health, and nutrition support, while enabling early identification and referral of vulnerable cases.

Child protection and psychosocial support remain central to the response. A total of 7,560 children accessed community-based psychosocial support, providing safe spaces for expression and emotional relief. In parallel, 65 children received case management and specialized services, while 208 adolescent girls participated in POWER4Girls emergency sessions.

Preventive and awareness-raising activities reached 8,595 individuals through child protection and justice for children's sessions, promoting positive coping strategies. Health and nutrition awareness sessions reached a further 9,801 children, adolescents, and women, supporting improved hygiene practices and health behaviours in emergency conditions.

Adolescent and youth engagement is contributing to resilience and social cohesion. A total of 604 adolescents participated in life skills and healthy lifestyle sessions, 368 benefited from non-formal skills training, and 114 youth actively engaged in community-led initiatives, including supporting learning and recreational activities for affected children.

Education support has reached 3,500 children following learning assessments, with children grouped by academic level to enable targeted support. Retention learning programmes are ongoing for 1,100 children aged 6–14, focusing on foundational skills and learning recovery. In parallel, early childhood education is provided to 2,400 children aged 3–5 through play-based and psychomotor approaches, supporting cognitive and social development.

Efforts to strengthen safe and accountable access to services included PSEA and AAP awareness sessions, reaching 12,998 individuals with information on available services and feedback mechanisms.

Supply and Logistics

Since the onset of the emergency, UNICEF has mobilized US\$ 6.2 million in procurement to support its multi-sectoral response in Lebanon. An airbridge between Copenhagen and Beirut, established with support from UNICEF Supply Division and partners including ECHO, the EU Humanitarian Air Bridge (EU HAB), and DSV Logistics, has enabled the rapid delivery of critical supplies. To date, two charter flights carrying 72 metric tons of medical and emergency supplies have arrived, with a third shipment of medical supplies expected to come with a EU HAB charter on 23 April.

In parallel, US\$ 6 million worth of prepositioned supplies, equivalent to 1,500 metric tons, have been dispatched since the beginning of the response. This includes US\$ 3.4 million in emergency supplies, comprising 103,885 emergency kits, 24,687 winter clothing kits, and 500,000 bottles of water, as well as US\$ 2.2 million in medical and nutrition supplies, alongside education and WASH materials. In addition, US\$ 5.7 million in stock, including emergency and medical supplies, is available in Beirut and three field locations (Wahle, Tripoli, and Ghazieh), helping to sustain the response over the coming weeks. Replenishment of emergency stocks is underway through local procurement, with 70,437 kits, including baby kits, hygiene kits, and dignity kits, currently in production.

In hard-to-reach areas, distributions continue through coordinated convoys. Additional service providers have been mobilized to support critical infrastructure and service continuity, including the installation of latrines and showers in shelters, fuel delivery to water establishments, and maintenance of water systems nationwide. To enhance operational safety and access, an additional armoured vehicle has been procured to support staff deployment to hard-to-reach and high-risk areas. UNICEF is also supporting the MoPH logistics operations at the Karantina Central Medical Store, with the deployment of a logistician contributing to warehouse and inventory management, as well as emergency distributions. Additional support and transport capacity are being provided to reinforce day-to-day operations.

Humanitarian Leadership, Coordination and Strategy

On 28 February 2026, MoSA, designated by the Government as the sole authority to coordinate with the Humanitarian Country Team (HCT) in the event of displacement, convened a meeting with Heads of UN Agencies to align on preparedness measures in anticipation of a potential escalation. During the meeting, MoSA confirmed that the agreed coordination architecture falls under the Lebanese Response Plan (LRP), through the Inter-Sector Coordination Group (ISCG) framework, led jointly by MoSA and the Ministry of Interior and Municipalities (MoIM). At the sub-national level, coordination is led by Governors in coordination with the National Disaster Risk Management Unit.

Within this framework, UNICEF co-leads the Education sector and leads the WASH and Nutrition sectors, as well as the Child Protection Working Group, at both national and sub-national levels. UNICEF also contributes to the Health, GBV and AAP, Gender, Livelihoods, and Basic Assistance sectors. UNICEF is also the co-lead of the Assessment and Analysis Working Group. At the sub-national level, the Operational Coordination Group (OCG) is coordinating the response in close collaboration with MoSA and Governors. UNICEF-led sectors are continuously monitoring partners’ emergency response capacities and contingency stock levels, while ensuring that sector-specific preparedness actions are undertaken in coordination with partners.

Funding Overview and Partnerships

UNICEF’s three-month Preparedness and Response Plan requires US\$ 48.3 million to reach 1 million people in need. There is US\$ 15.2 million in available funds for this response, leaving a funding gap of 69 per cent. In addition, UNICEF has advanced US\$ 6 million through its internal Emergency Programme Fund (EPF) to enable immediate response; this is a temporary measure and must be reimbursed once funding is received. Without additional resources, UNICEF will be unable to maintain critical services or respond to the escalating needs of affected children and their families.

Funding Needs	Funds Available	Funding Gap
\$48.3m	\$15.2m	\$33.1 (69%)

Communications and Advocacy



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Intensified Strikes on Lebanon and the Devastating Impact on Children

Ten-month-old Ahmad was injured in an Israeli airstrike that hit a nearby house in southern Beirut on 8 April.

Ahmad and his three-year-old brother, Khaled, were with their mother, Fatima, at a relative’s home when the strike hit a nearby building. The force of the blast shook the room, and the walls began to collapse. Fatima grabbed her two children and ran toward the door as smoke, dust, and debris filled the air. “It was apocalyptic; streets filled with cars, people screaming, bodies everywhere,” she said.

“No place feels safe for my children anymore,” Fatima said, sitting on the hospital bed and holding her two boys tightly. “Every sound makes Khaled jump and cry. I’m even afraid the hospital could be targeted,” she added, her voice trembling.

Media Interview from this Week Include:

- [UNICEF statement on intensified strikes on Lebanon and the devastating impact on children](#)
- [Teleticino TV Interview with Marcoluigi Corsi on latest escalation and situation of children](#)
- [CNN Interview with Christophe Boulhierac on situation following UNICEF statement](#)
- [BBC Interview with Christophe Boulhierac on situation following UNICEF statement](#)
- [Annahar Interview with Blanche Baz on situation of children](#)
- [ABC Interview with Christophe Boulhierac on situation and UNICEF support](#)

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